

Anti-Aging

Patient Education Handout

NAD+ · Metformin · KORB Health Group · For Patient Reference

This handout is for educational reference only. It is not medical advice. Always follow your prescription label and your KORB provider's guidance. If your label differs from anything here, follow your label and contact KORB.

What This Add-On Is

Two treatments aimed at how you age rather than at a specific symptom. They can be added to any KORB program, and you may be prescribed one or both. Neither requires lab work to start.

NAD+

NAD+ is a coenzyme your body already makes. It is involved in energy production, DNA repair, and immune function. Levels fall steadily with age, and supplementing aims to restore some of what has been lost.

Most people take it hoping for better energy, sharper focus, and less of the general worn-down feeling that creeps in with age. Research is promising but still developing, so think of it as supporting how you feel rather than producing a dramatic change.

How you take it	What to do
Injection under the skin	Twice a week. Most people use the lower abdomen; the arm, thigh, or buttock also work. Rotate the site.
Nasal spray	Once daily, one or two sprays in each nostril as directed.
Dissolving tablet	Under the tongue each morning, Monday to Friday, with weekends off. Let it dissolve fully — do not chew or swallow it whole.

Your NAD+ supply lasts 28 days, and you refill monthly rather than quarterly. If you use the injection, there will still be medication in the vial at 28 days. That is expected. Discard it and start your new vial. Compounded medication is not considered safe to use past that point once the vial has been punctured.

Tell your provider if you take isotretinoin, sold as Accutane. NAD+ is not used alongside it.

Metformin

Metformin has been used for decades for blood sugar and is on the World Health Organization's list of essential medicines. It is prescribed here for a different reason: a growing body of research links it to lower rates of dementia and some cancers, reduced cardiovascular risk, and less inflammation.

One extended-release tablet daily. Your supply is 90 days, so it follows the usual quarterly rhythm.

You will probably not feel any different, and that is normal. Unlike a medication that treats a symptom you can notice, metformin here is aimed at long-term risk. Not feeling a change does not mean it is not working.

You do not need to be diabetic to take it. At this dose it may nudge your blood sugar slightly lower, but it is unlikely to cause symptoms of low blood sugar.

Before you start, your provider needs to know about: kidney problems, liver problems, heart failure, or heavy alcohol use. Metformin is not appropriate for everyone, and these are the things that matter most. Also tell us if you are scheduled for surgery or an imaging scan that uses contrast dye, since metformin is usually paused around those.

Side Effects

What you may notice	What to do
Nausea or upset stomach	The most common effect for both. Usually settles. Taking metformin with food helps.
Headache or fatigue	May occur early on. Tell your provider if it persists.
Indigestion (NAD+)	Usually mild.
Dizziness, sweating, or headache together (metformin)	Can indicate blood sugar dipping. Have something to eat and tell your provider.
Injection site soreness or redness	Rotate sites and use new equipment each time.

Seek urgent care for: unusual muscle pain or weakness, trouble breathing, unusual sleepiness, severe stomach pain with nausea and vomiting, or feeling very cold or lightheaded. These are rare but are the symptoms that matter most while taking metformin. Do not wait to hear back from KORB.

Follow-Up

NAD+: every 28 days, since that is your supply length.

Metformin: every 90 days, alongside your other KORB visits.

No routine lab work is required for either, though your provider may check kidney function if you are on metformin.

Key Reminders

NAD+ refills monthly. Metformin refills quarterly.

Discard your NAD+ vial after 28 days even if medication is left.

Rotate injection sites and use new equipment every time.

You will probably not feel different on metformin. That is expected.

Tell your provider about kidney or liver problems, heart failure, or heavy alcohol use before starting metformin.

Tell your provider about upcoming surgery or a scan using contrast dye.

Tell your provider if you take isotretinoin (Accutane).

How to reach us

Operations, non-medical: +1 (888) 959-7299 · info@korbhealth.com · Mon–Fri, 9 AM–6 PM CT

Medical questions: message your provider at portal.kareo.com

Emergency: call 911 or go to the nearest emergency room.